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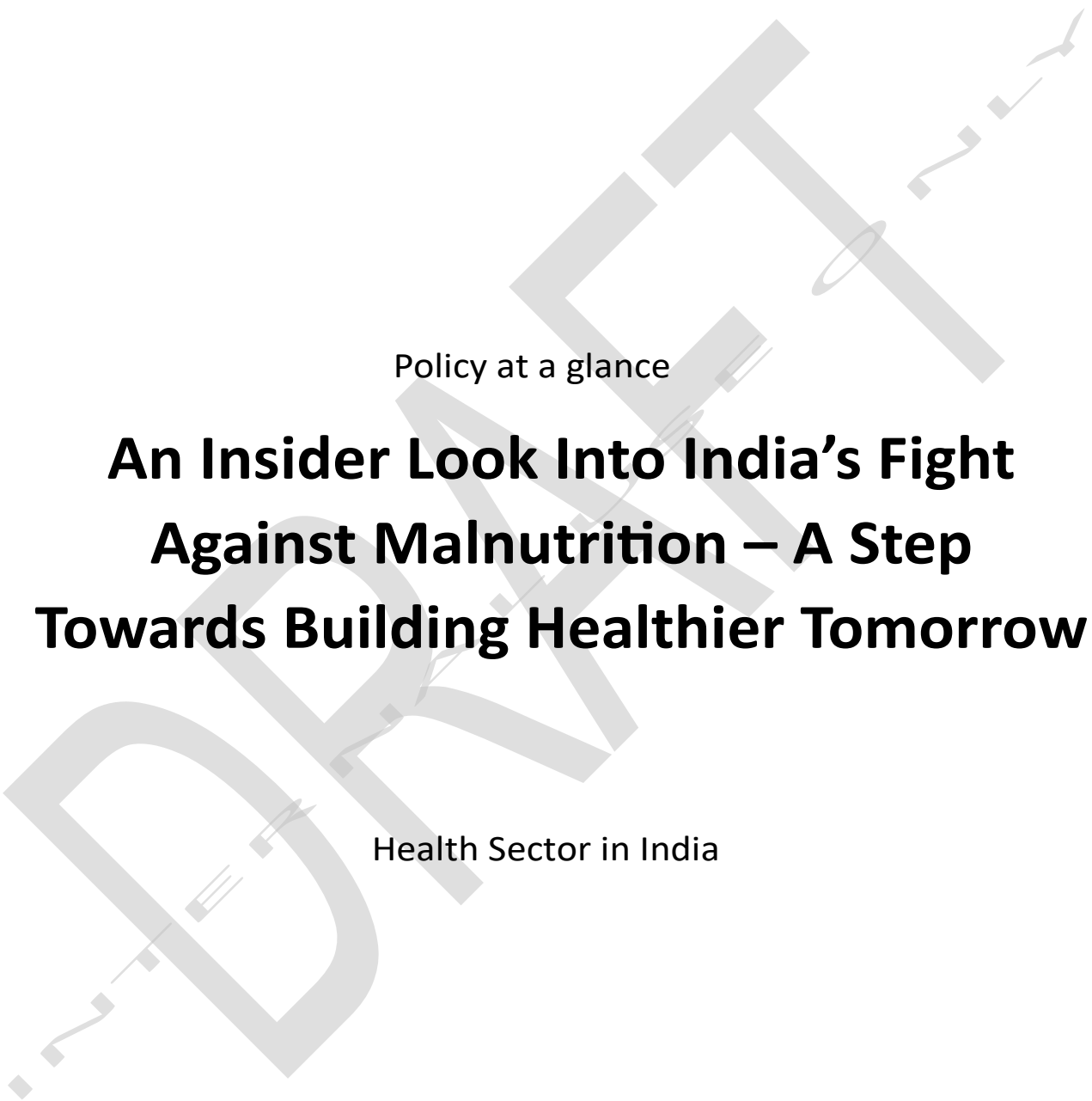


POLICY AT A GLANCE | 03

An Insider Look Into India's Fight Against Malnutrition – A Step Towards Building Healthier Tomorrow

Health Sector in India

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Policy at a glance

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The '**Policy-at-a-Glance**' provides users with curated information for first-level analysis and inter-state comparisons of policy. The document brings together all topic-relevant national and state-level acts, policies, and schemes in one place. Further, it identifies for a reader what policy features are covered in which document in a simple table.

The first section of this document sets out some '*Context and Background*' on the topic; the subsequent sections explain the '*Policy Features*' that have been compared; and the last section is the '*Table of Insights*' that marks out what '*Policy Feature*' has been covered (and on which page) in each of the topic-relevant documents.

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1. Context

There is a vital link between nutrition and overall health and development. And realizing its importance the Government of India (GoI) has been formulating policies and programmes pertaining to nutrition upliftment over the decades. This has led to the implementation of several comprehensive programmes aimed at combating malnutrition and improving nutrition across the nation. These efforts have particularly been focused on vulnerable populations (women, pregnant women, children, and adolescents), recognizing their heightened susceptibility to nutritional deficiencies.

Central government initiatives such as the National Nutrition Mission (Poshan Abhiyaan), Integrated Child Development Services (ICDS), and the Mid-Day Meal Scheme have been pivotal in addressing malnutrition and enhancing the nutritional status of children, pregnant women, and lactating mothers. Furthermore, the Ministry of Women and Child Development (MWCD) has recently launched a nationwide programme named Mission Saksham Anganwadi and Poshan 2.0 that focuses on improving nutrition through diet diversity, food fortification, and popularizing millets. Programmes such as Anaemia Mukht Bharat and Weekly Iron and Folic Acid Supplementation (WIFS) are also ongoing to support nutritional activities.

To improve the dietary quality of the vulnerable population, there is need to diversify food base and consider the regional, cultural and traditional food preferences. Government policies in India like POSHAN Atlas and Horticulture Mission can help in achieving this by identifying and producing region specific foods at the national level.

India has a rich diversity of indigenous foods, that can provide essential nutrients and help reduce hunger and malnutrition among Indian indigenous communities. Traditional rice varieties, which are eaten by indigenous communities in Jharkhand, Meghalaya, Himachal Pradesh and Arunachal Pradesh, had higher nutritional value than conventional varieties, according to a study. Similarly under-utilized legumes, such as Jack bean, Velvet bean, and Mucuna monosperma, which are eaten by indigenous communities in Kerala, had more protein, iron, calcium and zinc than staple legumes. (Kapoor et al.)

Nonetheless, the implementation of various specific features pertaining to the overall nutrition theme occurs on a state-wide basis, thereby yielding divergent levels of nutrition activities coverage across different regions of India. (United Nations. Food security and nutrition and sustainable agriculture. (n.d)). Highlighted below are some key features and practices that have contributed to the enhancement of nutrition levels.

This document collates various policy approaches aimed at enhancing nutrition across India. The government recognises the vital link between nutrition and overall health as it continues to focus on combating malnutrition. The state governments play a significant role in designing policies that enhance nutrition improvement schemes. Understanding the diverse approaches taken by different states is essential for further combating malnutrition.

2. Policy Features

States and Union Territories across the country have increased efforts to improve and enhance their nutrition improvement schemes so as to provide nutrition to more population groups and include more nutrition-rich products. Apart from national schemes like Mission Saksham Anganwadi and Poshan 2.0, several states have their own schemes to promote nutrition. (NITI Aayog. Take Home Ration: Good Practices Across The States/UTs. World Food Programme. 2022).

This section identifies a common set of policy features that document referred to in section 3 (table of insights) cover. The features are classified under the following three broad aspects of policy: 1) Standardised Diet, 2) Innovation, 3) Community Participation and IEC Activities.

1) Standardised Diet:

- a. **Millets in Public Distribution System (PDS) / Mid-Day Meal (MDM) / AWCs:** Millets are a valuable source of dietary fibre, promoting healthy digestion and averting constipation. They are naturally devoid of gluten,

making them a suitable choice for individuals with celiac disease. Millets also boast a high antioxidant content, safeguarding our cells against the harmful effects of free radicals. Additionally, their low glycaemic index assists in the effective management of diabetes. Because of these nutritional and health benefits, millets are included in the nutrition programmes of states like Odisha under its Odisha Millets Mission. (Department of Agriculture and Food Production. Odisha Millets Mission. n.d)

- b. **Eggs in MDM/AWC:** Eggs are a nutrient-rich food, offering a significant amount of protein, vitamins (A, B6, B12, and folate), essential amino acids, and minerals (iron, phosphorus, selenium, choline, and zinc) relative to their calorie content. The protein component of nutrition is one of the most important health indices that have a direct impact on a child's growth and development, which makes eggs an important component of a healthy diet. It is being provided in states like Andhra Pradesh under its YSR Sampoorna Poshana Plus Scheme. (Government of Andhra Pradesh. YSR Sampoorna Poshana Plus Scheme. n.d.)
- c. **Iron Folic Acid Tablets in PDS/MDM/AWC:** These supplements are given to address the significant issue of high anemia. The ultimate objective is to disrupt the cycle of anaemia that spans across generations. This is being provided in states like Himachal Pradesh under its Mukhya Mantri Bal Suposhan Yojana. (NHM. Mukhya Mantri Bal Suposhan Yojana. n.d.)

2) Innovation:

- d. **Technology Based Supply Chain Management:** Supply chain management (SCM) involves overseeing the movement of goods and services from source to destination. Technological advancements have revolutionized SCM by enhancing efficiency, reducing waste, and ensuring the establishment of transparent and efficient communication channels, as well as data analysis capabilities across the entire chain, thereby enhancing the effectiveness of the programme. This is currently being adapted by states like Himachal Pradesh under its Mukhya Mantri Bal Suposhan Yojana. (NHM. Mukhya Mantri Bal Suposhan Yojana. n.d.)

3) Community Participation and IEC Activities:

- e. **Community Based Events:** At the Anganwadi level, community-based gatherings are arranged for a single day to engage and educate young mothers and family members of children about nutrition. These events encompass a range of activities, including showcasing diverse food recipes and preparing various recipes live to instruct mothers; and also raising awareness on good health and nutrition practices. This is being followed in states like Haryana under the Haryana Poshan Abhiyan. (NHM. POSHAN ABHIYAN Haryana. n.d.)
- f. **Training of Anganwadi Workers (AWW) and Self-Help Groups (SHGs) on Nutrition:** AWWs and SHGs receive nutrition training to empower them as valuable resources for advocating nutrition within their communities. They actively contribute in raising awareness about the significance of good nutrition during their weekly meetings. This is seen in like Jammu and Kashmir under its ICDS program. (Department of Social Welfare Jammu and Kashmir. Poshan Abhiyan. n.d)

Nutrition policies and programmes in India are the cornerstone of a healthier, more prosperous, and equitable future for our nation. By prioritizing the well-being of citizens through comprehensive, state-specific nutrition interventions, malnutrition can be eliminated and public health can be improved. States like Karnataka, Maharashtra, Telangana, Tamil Nadu are front-runners in actively formulating multiple nutrition specific interventions.

3. Table of Insights: Policy Initiatives for Nutrition

Note 1: Documents linked in the tables include original Acts, Policies, Schemes, Programmes and Guidelines. It is aimed at highlighting and enabling a comparison of relevant policy features covered in the documents, across the centre and the states. While all important documents have been identified, the coverage may not be exhaustive. Further, the 'Policy-at-a-Glance' is not aimed at analysing nuances of policies.

Note 2: Page numbers specified in the Table of Insights are as per the PDF pagination opened in a Google Chrome browser. It is possible to encounter differences in the actual page number for some of the documents when opened in a different web browser/application.

Geography	Data Points					Policy References	Standardised Diet			Innovation Technology-Based Supply Chain Management	Community Participation and IEC Activities	
	<u>Children under 5 years who are Stunted (Height-for-age) (%)</u>	<u>Children under 5 years who are Wasted (Weight-for-Height) (%)</u>	<u>Any Anaemia in Women (%)</u>	<u>Any Anaemia in Men (%)</u>	<u>Any Anaemia in Children (% of children age 6-59 months) (%)</u>		Millet in PDS / MDM / AWCs	Eggs in MDM/AWC	Iron Folic Acid tablets in PDS/MDM/AWC		Community-Based Events	Training of Anganwadi Workers (AWW) and Self-Help Groups (SHGs) on Nutrition
Centre	36	19	57	25.1	67.1	Mission Saksham Anganwadi and Poshan 2.0	Yes (Pg. 23)		Yes (Pg. 20)		Yes (Pg. 35)	Yes (Pg. 41)
						ANDAMAN AND NICOBAR ADMINISTRATION DIRECTORATE OF SOCIAL WELFARE		Yes (Pg. 3)				
Andhra Pradesh	31.2	16.1	58.8	16.3	63.2	Women Development and Child Welfare Department - Andhra Pradesh		Yes (Pg. 11)				
						Arunachal Scheme Handbook			Yes (Pg. 177,183)			
Assam	35.3	21.7	65.9	36	68.4	SCHEME FOR ADOLESCENT GIRLS (SAG), Administrative Guidelines - Assam			Yes (Pg. 3)			

Geography	Data Points					Policy References	Standardised Diet			Innovation Technology-Based Supply Chain Management	Community Participation and IEC Activities	
	<u>Children under 5 years who</u>	<u>Children under 5 years who</u>	<u>Any Anaemia</u>	<u>Any Anaemia in Men (%)</u>	<u>Any Anaemia in Children</u>		Millet in PDS / MDM / AWCs	Eggs in MDM/AWC	Iron Folic Acid tablets in		Community-Based Events	Training of Anganwadi Workers (AWW)

	<u>are Stunted (Height-for-age) (%)</u>	<u>are Wasted (Weight-for-Height) (%)</u>	<u>in Women (%)</u>		<u>(% of children age 6-59 months) (%)</u>				PDS/MDM/AWC			and Self-Help Groups (SHGS) on Nutrition	
Dadra & Nagar Haveli and Daman & Diu	39.4	21.6	62.5	24.6	75.8	4C Project Beti Janmahotsav			Yes				
Goa	25.8	19.1	38.9	12	53.2	Central Schemes - Goa			Yes				
Gujarat	39	25.1	65	26.6	79.7	Women and Child Development Department - Gujarat			Yes				
						Best Practices Women and Child Development Sector			Yes				
Haryana	27.5	11.5	60.4	19	17.4	Poshan Abhiyan - Haryana					Yes		
Himachal Pradesh	30.8	17.4	53	18.6	55.4	MUKHYA MANTRI BAL SUPOSHAN YOJANA - Himachal Pradesh		Yes (Pg. 29)		Yes (Pg. 30)	Yes (Pg. 42)	Yes (Pg. 54)	
Jammu & Kashmir	26.9	19	65.9	36.7	72.7	Mission Poshan - Department of Social Welfare - J & K						Yes	
Geography	Data Points					Policy		Standardised Diet			Innovation	Community Participation and IEC Activities	
	<u>Children under 5 years who are Stunted (Height-for-age) (%)</u>	<u>Children under 5 years who are Wasted (Weight-for-Height) (%)</u>	<u>Any Anaemia in Women (%)</u>	<u>Any Anaemia in Men (%)</u>	<u>Any Anaemia in Children (% of children age 6-59 months) (%)</u>	Reference	Millets in PDS / MDM / AWCs	Eggs in MDM/AWC	Iron Folic Acid tablets in PDS/MDM/AWC	Technology-Based Supply Chain Management	Community-Based Events	Training of Anganwadi Workers (AWW) and Self-Help Groups (SHGS) on Nutrition	

Karnataka	35.4	19.5	47.8	19.6	65.5	INTEGRATED CHILD DEVELOPMENT SERVICES SCHEME, Karnataka		Yes	Yes		Yes	Yes
Kerala	23.4	15.8	36.3	17.6	39.4	Department of Women and Child Development, FIRST 1000 DAYS			Yes			
Ladakh	30.5	17.5	92.8	75.6	93.9	Integrated Child Development Service - Kargil			Yes			
Maharashtra	35.2	25.6	54.2	21.9	68.9	Women and Child Development Department - Maharashtra (Kishori Shakti Yojana)			Yes			
						SCHEME FOR ADOLESCENT GIRLS (SAG) - Maharashtra			Yes (Pg. 5)			
						Indira Gandhi MatritvaShayogYojna (IGSMY)			Yes (Pg. 3)			
Meghalaya	46.5	12.1	53.8	25.5	45.1	Mid-Day Meal Scheme - Meghalaya						Yes
Geography	Data Points					Policy	Standardised Diet			Innovation	Community Participation and IEC Activities	
	Children under 5 years who are Stunted (Height-for-age) (%)	Children under 5 years who are Wasted (Weight-for-Height) (%)	Any Anaemia in Women (%)	Any Anaemia in Men (%)	Any Anaemia in Children (% of children age 6-59 months) (%)	Reference	Millet in PDS / MDM / AWCs	Eggs in MDM/AWC	Iron Folic Acid tablets in PDS/MDM/AWC	Technology-Based Supply Chain Management	Community-Based Events	Training of Anganwadi Workers (AWW) and Self-Help Groups (SHGS) on Nutrition
Odisha	31	18.1	64.3	28.5	64.2	Odisha Millets Mission	Yes					

						Department of Women and Child Development and Mission Shakti, Mo-Chhatua initiative						Yes
Tamil Nadu	25	14.6	53.4	15.2	57.4	Puratchi Thalavir M.G.R. nutritious meal programme - Mid-Day Meal Scheme		Yes				
						Puratchi Thalavir M.G.R. nutritious meal programme, SCHOOL HEALTH PROGRAMME			Yes			

Geography	Data Points					Policy	Standardised Diet			Innovation	Community Participation and IEC Activities	
	Children under 5 years who are Stunted (Height-for-age) (%)	Children under 5 years who are Wasted (Weight-for-Height) (%)	Any Anaemia in Women (%)	Any Anaemia in Men (%)	Any Anaemia in Children (% of children age 6-59 months) (%)	Reference	Millets in PDS / MDM / AWCs	Eggs in MDM/AWC	Iron Folic Acid tablets in PDS/MDM/AWC	Technology-Based Supply Chain Management	Community-Based Events	Training of Anganwadi Workers (AWW) and Self-Help Groups (SHGs) on Nutrition
Telangana	33.1	21.7	57.6	15.3	70	Department of Women Development and Child Welfare, Supplementary		Yes				

						Nutrition Programme(SNP)						
						Department of Women Development and Child Welfare, Arogya Lakshmi Programme			Yes			
						Department of Women Development and Child Welfare, Special care and supervised feeding		Yes				
						Department of Women Development and Child Welfare, Balamrutham		Yes				

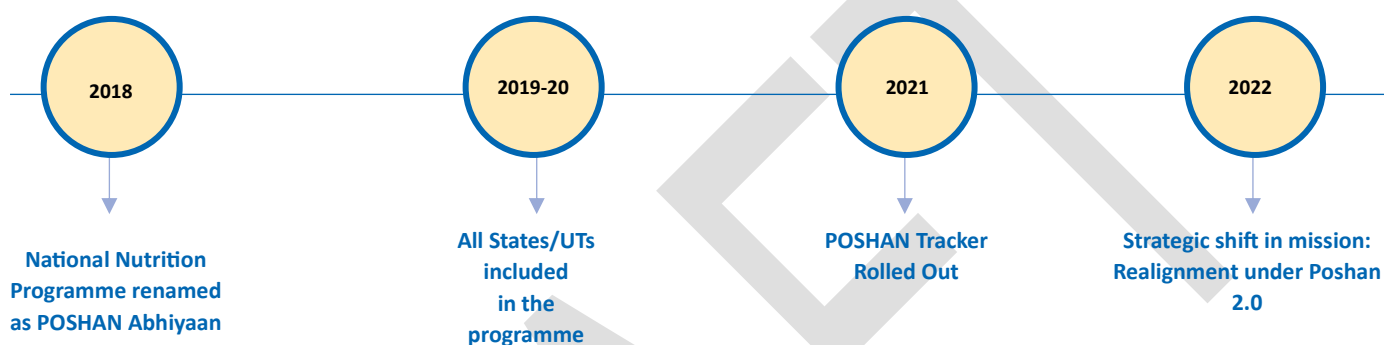
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4. Additional Information

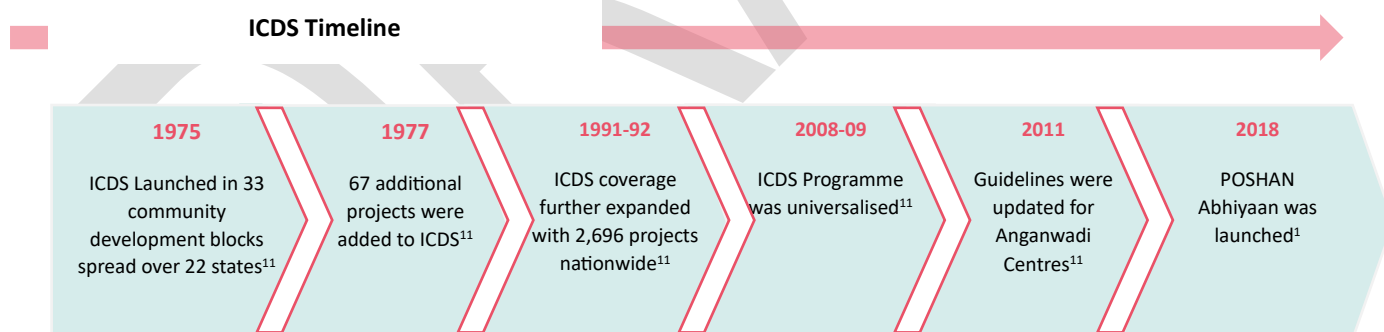
Implementation of Poshan Abhiyan: The Prime Minister launched POSHAN Abhiyaan, a mission mode scheme for holistic nutrition, to achieve the goal of 'Suposhit Bharat'. In the Budget 2021-2022, Mission Poshan 2.0 (Saksham Anganwadi and Poshan 2.0) was announced as an integrated nutrition support programme that aims to improve the nutritional quality, delivery, outreach and outcomes of health and wellness services.

It merges Anganwadi Services Scheme, Supplementary Nutrition Programme, Poshan Abhiyan and modified Scheme for Adolescent Girls. Poshan 2.0 targets Maternal Nutrition, Infant and Young Child Feeding Norms, Treatment of MAM/SAM and Wellness through AYUSH practices to reduce the prevalence of wasting, under-weight, stunting and anaemia.

Timeline of Implementation

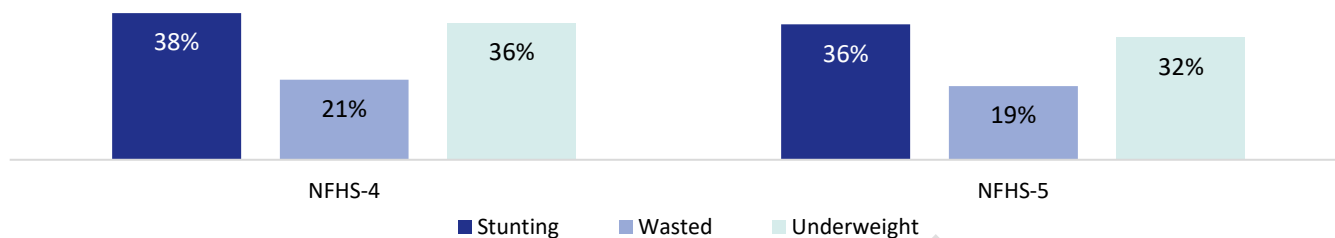


Integrated Child Development Scheme (ICDS): ICDS is one of India's oldest and most extensive programmes for children and women. It was launched in 1975 as a response to the prevailing issues of malnutrition, maternal and child mortality, and poor childcare practices. Integrated Child Development Services aims to provide a package of services that includes supplementary nutrition, immunisation, healthcare, and preschool education to children below the age of six and pregnant and lactating mothers.



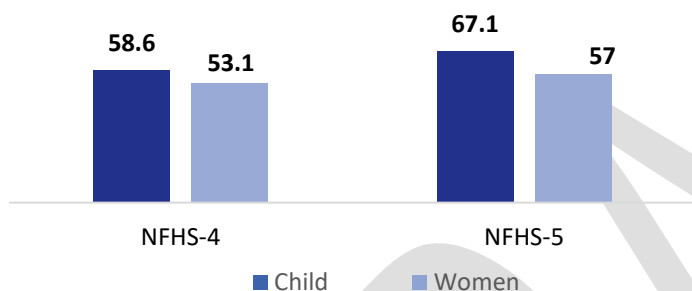
Key Nutrition Issues: Indian children suffer from triple burden of stunting, wasting, and underweight. 19% of children within the age group of 0 to 5 years are still suffering from wasting as per NFHS-5 survey.

Nutritional Status of Children

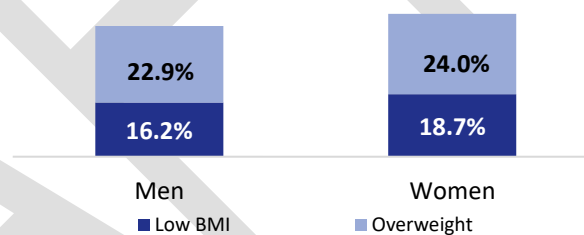


Anaemia prevalence among women aged (15-49 years) grew from 53.1% in 2015-2016 to 57% in 2019-2021. 67.1% of children (6 months - 59 months) were found to be anaemic in the National Family Health Survey-5 (NFHS-5). 22.9% and 24% population of men and women were found to be overweight in NFHS-5 survey, which is gradually becoming a rising concern along with malnutrition.

Anaemia Prevalence in %



NFHS-5 Overweight & Low BMI in %



List of Acronyms

- ICDS: Integrated Child Development Services
- MWCD: Ministry of Women and Child Development
- WIFS: Weekly Iron and Folic Acid Supplementation
- WSHGs: Women Self Help Groups
- SCM: Supply chain management
- AWW: Anganwadi Workers
- AWCs: Anganwadi Centers
- PDS: Public Distribution System
- MDM: Mid-Day Meal
- THR: Take Home Ration
- SAG: Scheme For Adolescent Girls
- SNP: Supplementary Nutrition Programme
- NFHS: National Family Health Survey
- GoI: Government of India

6.

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